

Name: نادية ابراهيم بيومي احمد
National ID: 26212162101884



تقرير طبي

السيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بان نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
--:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي IV (trastuzumab emtansine) TDM1

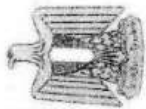
الوقت :

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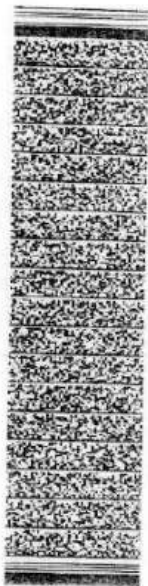
التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين



٢٠٢٢/١٠ ٢٦٢١٢١٦٢١٠١٨٨٤
 بدون عمل
 أنثى مسلمة
 محمد خالد عامر أبو الذهب
 البطاقة سارية حتى ٢٠٢٩/١٠/٠٨



٢٠٢٢/١٠
 ٢٦٢١٢١٦٢١٠١٨٨٤

مملكة البحرين
 بطاقة تطبيق الهوية الوطنية

نادية
 ابراهيم بيومي احمد
 ١٦٦ ش سعد زغلول
 الحوامدية - الجيزة

٢٦٢١٢١٦٢١٠١٨٨٤



JP3141153

٢٠٢٢/١٠
 ٢٦٢١٢١٦٢١٠١٨٨٤

جمهورية مصر العربية
بطاقة تحقيق الشخصية

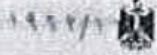


نلايه

ابراهيم بيومي احمد

١٦٦ سن زغول

الحوامدية - الجيزة



JP3141153

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٢٠٢٢/١٠ ٢١٢١٢١٦٢١٠١٨٨٤

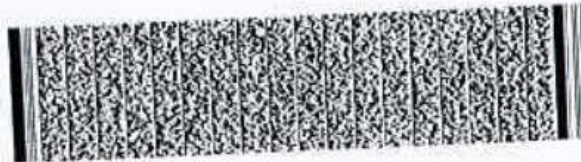
بدون عمل



انثى مسلمة

متزوجة محمد خالد عامر ابو الذهب

البطاقة سارية حتى ٢٠٢٩/١٠/٠٨



Patient Name: Nadia Ibrahim Bayoumi Ahmed	Date : 29/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 62 Year(s)
Medical ID: 1687/2025	Pat. ID: 99666

TRIPHASIC MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Known case of RBC

I Chest:-

- Right breast small nodule RIQ 3.6 x2.5 cm and smaller nodule 12 x11 mm
- Left breast mass 7.2 x5.7 likely benign (most of left breast is out of field of examination).
- Bilateral enlarged axillary L. Ns , the largest at right axilla 18 x8 mm.
- Small pretracheal L. N 6mm.
- No sizable pulmonary nodules or masses.
- No evidence of pleural effusion, pleural thickening or masses seen.
- No gross cardiac abnormality. No pericardial effusion detected.
- Normal appearance of the chest wall.

II Abdomen & pelvis:

- The liver is mild enlarged in size with homogenous CT texture. No gross focal lesions.
- Mild enlarged spleen with normal parenchymal texture of the , no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, and both suprarenals.
- Few subcentimetric mesenteric L. Ns.
- Normal appearance of the other visualized pelvic organs.
- No abdominal or pelvic masses or collections.

III.Scanned skeleton:

- Small sclerotic lesions of left femoral head and posterior left acetabular column.

Opinion:

- Right breast mass with axillary lymphadenopathy.

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Medical ID: 1687/2025	Pat. ID: 99666

- Small sclerotic lesions of left femoral head and posterlor left acetabular column likely bone island for correlation with bone scan.
- No evidence of hepatic or pulmonary metastatic deposits.

DR/Reham Khedr,,,MD

29/7/2025

Not Approved

Not Approved



مملكة العربية السعودية
وزارة الصحة العامة
الهيئة العامة للغذاء والدواء



07/01/2024

MDT FORM

Patient Name: <u>Maria Ibrahim Ezzamel Ahmed</u>		MRN: <u>1148</u>	
Age: <u>54 Y</u>	Sex: <u>F</u>	ID: <u>2021216210284</u>	Phone: <u>01122233442</u>
Menopausal Status: <u>Yes</u>		Medical Status: <u>Family History</u>	
NRS SCORE: <u>0</u>		Reason for MDT: <u>POST OP</u>	
CO Morbidity: <u>DM HTN</u>			

Clinical Data:

DX:

EC: R PR: S/N HER2: 3 Ki67: 18

Sentinel lymphography:

Metastasis:

History:

POST MACT, POST OP
RT DM+SCAB
HISTOLOGICAL VIABLE IDC G 2
SLN (3/3)
FREE DEEP MARGIN
PNE
TIC N1

Recommendations:

MDT Outcome & Plan

1. POST to RT DM A, PCLN
2. Adjuvant tamoxifen x 12 cycles (on day MCH protocol)
3. Adjuvant ET (AI, at least 2 yrs)
4. Zolmita Q 6 mths.

TNM



SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	NEUTRITION	PSYCHIATRY

Patient Name :	الاستاذ / نادية ابراهيم بيومي احمد	Request No. :	13006388
Patient No. :	26212162101884	Request Date :	07/05/2026
Path No. :	1396/26	Verification Date :	24/05/2026
Sex :	Female	Financial Category :	جوستاف روسي - مبادره
Age :	64 Y	Department :	Out Patient
Referreing Doc :		Registered by :	Ali mostafa

Pathology Report

Clinical Data: Right breast cancer (Post-NACT).

Nature of specimen: Right simple mastectomy & Sentinel lymph nodes biopsy.

Gross Examination:

- * Right simple mastectomy specimen measured 30x20x5 cm, with covering skin ellipse 25x18 cm. Nipple & areola looked unremarkable grossly.
- * Serialling showed a tumor bed site measured 7x6 cm, located 0.7 cm from the least deep margin.
- * Sentinel lymph nodes: Fibrofatty tissue piece measured 4x3 cm, entangling 9 (Only 4 submitted for frozen) lymph nodes.
- * 12 Blocks:
1:7- TBS., 8- Nipple., 9- Deep margin ., 10:12- SLNs.
- Gross was done by; Dr. Heba El Shafeey.

** Frozen (Intra-operative) results:

- SLNs: Four free sentinel lymph nodes (0/4).

Microscopic Examination:

- * Histological Type: Residual invasive duct carcinoma
- * Actual tumor size: 1.3 cm Cellularity (25%) .
- * Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
- * Angioinvasion: Absent.
- * Ductal carcinoma in situ: 1%, low grade DCIS, Solid pattern
- * Microcalcification: Present.
- * Nipple & areola: Free.
- * Deep margin: Free.
- * Sentinel lymph nodes: Positive metastatic deposits in three out of nine sentinel lymph nodes (3/9) (The largest metastatic focus measuring 0.4 cm, without extra-nodal extension).

Verified by :

Dr Maha Guimie

Printing Date : 24/05/2026 10:26:02

Page 1 of 2

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / نادية ابراهيم بيومي احمد
Patient No. : 26212162101884
Path No. : 1396/26
Sex : Female
Age : 64 Y
Referreing Doc :

Request No. : 13006388
Request Date : 07/05/2026
Verification Date : 24/05/2026
Financial Category : جوسفاف روسي - مبادره
Department : Out Patient
Registered by : Ali mostafa

Pathology Report

* Histologic response to therapy: Pathological partial response to therapy.

Diagnosis:

- Right breast simple mastectomy & Sentinel lymph nodes biopsy, Post NACTH
- Residual viable invasive duct carcinoma, Grade 2.
 - Sentinel lymph nodes: Positive metastatic deposits in three out of nine sentinel lymph nodes (3/9)
 - Deep margin: Free.
 - Pathological partial response to therapy.
 - Stage: ypT1c, ypN1a.

Signature

Dr.Walaa Ibrahim Ghoneim

Verified by :
Dr Maha Guimie

Printing Date : 24/05/2026 10:26:02

Page 2 of 2

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name : الأستاذ / نادية ابراهيم بيومي احمد	Request No. : 13004595
Patient No. : 26212162101884	Request Date : 06/04/2025
Path No. : 1193/25	Verification Date : 19/04/2025
Sex : Female	Financial Category : اورام دار السلام - مبادره
Age : 63 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data:

Bilateral breast lesions

Nature of the specimen:

Tru cut biopsies, collected at 6/4/2025 .

Gross Examination:

- (1) Right breast: Multiple greyish white tissue cores up to 1 cm , totally submitted.
- (2) Left breast: Multiple greyish white tissue cores up to 0.5 cm , totally submitted.

Microscopic examination:

- (1) Cores examined reveal diffuse marked stromal fibrosis with focal hyalinization and focal adenosis with usual ductal hyperplasia. Some of the proliferating ducts are irregularly dilated. The ducts are lined by inner columnar and outer myoepithelial cells. There is no evidence of malignancy in sections examined.
- (2) Cores examined reveal benign growth made up of proliferating small patent mammary ducts. Some of the ducts are compressed and closed, other are patent. The ducts are lined by inner columnar and outer myoepithelial cells. The stroma in between shows marked hyalinization. There is no evidence of malignancy in sections examined.

Diagnosis:

Right breast lesion, tru cut biopsy:

- Picture is suggestive of fibrocystic disease of the breast.

Left breast lesion, tru cut biopsy:

- Fibroepithelial lesion keeping with fibroadenoma with marked hyalinization.
- The case is for clinical and radiological correlation.

Dr. Marwa Mahmoud

Verified by :

dr.esraa magdy



Patient Name : (الاسم / مذكبة ابراهيم بنو من احمد)	Request No. : 11307008
Patient No. : 26212162101884	Request Date : 22/05/2025
Path No. : 1863/25	Verification Date : 10/06/2025
Sex : Female	Financial Category : (الفئة المالية - مبداء)
Age : 63 Y	Department : Out Patient
Referring Doc :	Registered by : amarwa mohamed you

Pathology Report

Clinical Data

Right breast mass

Nature of specimen

Tru cut biopsy, collected at 22/5/2025, 10:30 am

Gross examination

Multiple grayish white tissue cores up to 1.5 cm, totally submitted.

IHC technique: Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), Ki67 clone (30-9)

Microscopic examination

- * Histological type: Invasive duct carcinoma
- * Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade II
- * DCIS: Focal
- * Angioinvasion: Couldn't be detected in the received biopsy
- * Perineural invasion: No
- * IPT (Positive control showed appropriate reactivity) :
 - * ER: Strong nuclear reaction in 100% tumor cells (Allred score 9/8)
 - * PR: Strong nuclear reaction in 2% tumor cells (Allred score 5/6)
 - * HER2: Strong complete membrane reaction in >10 % of tumor cells (score 3+)
 - * Ki 67: 18 % nuclear reaction of tumor cells

Diagnosis *

Right breast mass, Tru cut biopsy:

Verified by :

Dr.Heba Hasan

Patient Name : الأستاذة / نادية إبراهيم تيمس أحمد
Patient No. : 26212162101884
Path No. : 1863/25
Sex : Female
Age : 63 Y
Referring Doc :

Request No. : 13007038
Request Date : 22/05/2025
Verification Date : 10/06/2025
Financial Category : Out Patient
Department : Out Patient
Registered by : marwa mohamed youssef

Pathology Report

Invasive duct carcinoma, Grade II

IPT:

- * ER: Positive (score 8/8).
- * PR: Positive (score 5/8).
- * HER2: Positive (score 3+).
- * Ki67: Positive in 18% of tumor cells.

Signature

Dr. Walaa Ghoneim

Dr.Heba Hassan

Verified by :
Dr.Heba Hasan



Patient Name: Nadia Ibrahim Bayoumi Ahmed	Date : 17/05/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 62 Year(s)
Medical ID: 26212162101884	Pat. ID: 99666

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Patient is complaining of BILATERAL BRESAT MASSES

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- The breasts are Scattered fibro glandular density (ACR B)
- **Left breast:**
- left breast UOQ mass of mixed density measured 8x6cm CORRESPONDING TYO LEFT 1 to 3 o'clock mass of mixed ecgcity.
- left UOQ peri areolar circumscribed oval shaped mass lesion of equal density by US corresponding to left 1 o'clock periareolar hypoechoic poorly vascular lesion measured 3x1.5cm .
- **Right breast:**
- Right upper central hyperdense mass with architectural distortion and few microcalcifications
- BY US seen as right 12 o'clock alerted area of hypoechogenic mounted to irregular mass measured 4.8x2.7 cm seen at 1 cm from skin and showing anterior dilated ducts reaching nipple.
- Right 12 o'clock paraareolar well hypoechoic poorly vascular lesion measured 2.5x1.6cm .
- Normal skin thickness and nipple-areola complex.
- **Nonspecific axillary LNs bilaterally**
- Clear sup[ran ad infraclavicular regions

Opinion:

- Right breast 12 o'clock highly suspicious area of alerted parenchyma mounted to irregular mass for rebiopsy BIRDA54C
- No pathological right axillary LNs.
- left breast UOQ large hamartoma BIRADS2
- Left breast left 1 I 'clock periareolar and right breast 12 o'clock peraireolar probably benign mass lesion likely fibroadenoma (BIRADS 3), for follow up ultrasound after 6month.

Dr ,BASMA MOHMAED

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DR/Reham Khedr,,,MD

29/7/2025

Not Approved

Not Approved

ECHO REPORT

NAME:

أحمد بن محمد / DATE: 29 / 9 / 2026

ID NUMBER:

SEX: AGE:

EF	60%	55-75%	PWT	1.2	0.6-1.1cm
LVEDD	4.0	3.5-5.8cm	AO.R		3-3.7cm
LVESD	3.1	2-4cm	LA	4.1	2-4cm
FS	30	25-45%	RV		0.7-2.3cm(supine)
SWT	1.2	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions

LV Wall Thickness

LV Diastolic Function

SWMA

LA and Ao Root Dimensions

Mitral Valve

Aortic Valve

Tricuspid Valve

Pulmonary Valve

Right Side and Systolic Function

Intracardiac Masses and Effusion

DR